

# Left knee MRI - 25 September 2026

Report transcription; original PDF unavailable.

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TRANSCRIPTION of the report supplied by the patient; original PDF unavailable.

### Clinical history

Assess cause of FFD and progress of semimembranosus oedema on previous scan

### Technique

Multiplanar, multisequence MRI without gadolinium.

### Comparison

No prior study for comparison.

### Findings

Cruciate ligaments are intact.

No meniscal tear. Collateral ligaments are intact. Medial and lateral compartment articular cartilage surfaces are well-maintained with no established osteoarthritic change.

Patellofemoral joint is normally aligned with preserved articular cartilage surfaces. Extensor mechanism tendons and iliotibial band insertion are intact.

No joint effusion or intra-articular loose body. No Baker's cyst. No concerning bone lesion or soft tissue mass. The major neurovascular structures are structurally normal.

There is a focal area of low-grade muscle oedema within the distal muscle belly of the semimembranosus at the distal myotendinous junction, suggesting a subacute low-grade strain. No discrete haematoma or tear defect. The distal free tendon and its distal insertion are intact and of normal signal intensity.

The remainder of the muscles and tendons around the knee joints are normal.

### Conclusion

Mild oedema within the distal muscle belly of semimembranosus, at the distal myotendinous junction, suggestive of a subacute low-grade muscle strain. The distal tendon is intact and inserts normally.

No internal joint derangement.

## MRI RIGHT HAND/WRIST

### Clinical History:

Check for microfracture versus pseudogout.

### Technique:

Multiplanar, multisequence MRI without gadolinium.

### Comparison:

Peroneus CT scan from 14 November 2025.

### Findings:

DISTAL RADIOULNAR JOINT: No active synovitis. No osteoarthritis.

RADIOCARPAL COMPARTMENT: No active synovitis. No osteoarthritis.

MIDCARPAL COMPARTMENT: No active synovitis. No osteoarthritis.

CARPOMETACARPAL JOINTS: No active synovitis. No osteoarthritis.

METACARPOPHALANGEAL JOINTS: No active synovitis. No osteoarthritis.

INTERPHALANGEAL JOINTS: No active synovitis. No osteoarthritis.

BONES: No osteitis or bone erosions. No fracture. No concerning incidental bone lesion.

FLEXOR TENDONS: Intact and of normal signal intensity. No active tenosynovitis.

EXTENSOR TENDONS: Intact and of normal signal intensity. No active tenosynovitis.

OTHER/INCIDENTAL: Small incidental ganglion cysts present along the volar aspect of the 4th metacarpophalangeal joint.

### CONCLUSION:

On this unenhanced MRI, there are no features to suggest synovitis, joint effusion or significant osteo arthritic changes. No osteitis or bony erosions.

No features to suggest tenosynovitis. No deposition disorder.

## XRAY THORACIC AND LUMBAR SPINE

### Clinical History:

check for crush fractures

### Findings:

No thoracolumbar crush fracture is present. Disc heights are preserved without significant degenerative change. No bony neuroforaminal stenosis is identified.

# Bone mineral densitometry - 16 April 2026

Original report excerpt; identifying details removed.

## EXAMINATION:

### BONE MINERAL DENSITOMETRY

HISTORY: Family history of osteoporosis, possible hypophosphatasia

No falls in last 12 months. No minimal trauma fractures.

TECHNIQUE: Dual Energy X-Ray Absorptiometry (DEXA) of the lumbar spine and femoral neck were performed on the Hologic scanner. Diagnosis usually based on Z score for this age group.

FINDINGS: The mean areal Bone Mineral Densitometry of the lumbar spine ( L1-L4, AP projection) is 0.766 g/cm<sup>2</sup> which lies 3.0 standard deviations below the mean for age (Z score).

The mean areal Bone Mineral Densitometry of the left femoral neck is 0.547 g/cm<sup>2</sup> which lies 2.6 standard deviations below the mean for age (Z score).

The mean areal Bone Mineral Densitometry of the left total hip is 0.717 g/cm<sup>2</sup> which lies 2.0 standard deviations below the mean for age.

## CONCLUSION:

Today's examination demonstrates bone density at or greater than 2 standard deviations below the mean for age in keeping with low bone mass. Follow up post treatment would be of value.

## EXAMINATION:

### DOPPLER ULTRASOUND LEFT LOWER LIMB VEINS

## HISTORY:

Varicose veins left leg To rule out DVT.

## FINDINGS :

The deep and superficial veins in the left lower limb are patent and demonstrate normal flow and compressibility. There is no deep vein thrombosis or superficial vein thrombosis.

No Baker's cyst is identified on the left side.

## CONCLUSION:

No evidence of deep vein thrombosis or superficial thrombophlebitis.

No Baker's cyst or other abnormality identified.

CT BILATERAL HANDS, LEFT ELBOW, RIGHT KNEE, RIGHT ANKLE

Clinical History:

Treated for gout.? Any urate crystals.

Technique:

Dual energy CT bilateral hands, left elbow, right knee, and right ankle.

Comparison:

MRI right knee 13 September 2025.

Bilateral hand x-rays 10 November 2023.

Findings:

Dual energy images demonstrate no urate crystal deposition involving bilateral hands, left elbow, right knee, or right ankle. No soft tissue calcification or osseous erosions identified. No significant joint effusion identified.

CONCLUSION:

No urate crystal deposition identified.

EXAMINATION:

ULTRASOUND BILATERAL HANDS

HISTORY:

Painful flexion of the digits

? cause

Gout

? fibrosis

FINDINGS:

Right Hand

Thumb:

The flexor and extensor tendons are intact and normal in appearance. No joint effusion and no other soft tissue abnormality.

Left Hand

Thumb:

The flexor and extensor tendons are intact and normal in appearance. No joint effusion and no other soft tissue abnormality.

Left index Finger:

The flexor and extensor tendons are intact and normal in appearance. No joint effusion and no other soft tissue abnormality.

Left middle Finger:

The flexor and extensor tendons are intact and normal in appearance. No joint effusion and no other soft tissue abnormality.

CONCLUSION:

No soft tissue depositions to suggest crystal deposition disease. No evidence of tenosynovitis or tendon tear. The cause of the patient's symptoms has not been identified.

## ULTRASOUND MSK INTERVENTIONAL

### Clinical History:

Tendinosis of the semimembranosus.

### Findings:

1 mL of Celestone and 3 mL of Marcaine has been injected superficial to the tendinosis of the semimembranosus of the right knee. Subsequently fenestration of the inflamed portion of the tendon was also performed. Procedure uneventful.

# Right knee MRI - 13 September 2025

Original report excerpt; identifying details removed.

## MRI RIGHT KNEE

### Clinical History:

Acute on chronic knee injury. Knee swelling while doing hamstring stretch. Posterior knee pain and medial aspect. Damage? Baker's cyst?

### Technique:

Multiplanar, multisequence MRI without gadolinium.

### Comparison:

No prior study for comparison.

### Findings:

Cruciate ligaments are intact.

No meniscal tear. Collateral ligaments are intact. Medial and lateral compartment articular cartilage surfaces are well preserved.

Patellofemoral articular cartilage surfaces are well-maintained. No chondromalacia patella. Extensor mechanism tendons and iliotibial band insertion are normal. No prepatellar soft tissue oedema or bursitis.

There is marked insertional tendinosis of semimembranosus at the tibial insertion, associated with a small ganglion cyst and peritendon soft tissue oedema. The gastrocnemius origins are intact. Biceps femoris and pes anserine tendons are intact. Normal popliteus tendon.

No significant joint effusion. Minimal knee joint effusion. No intra-articular loose body. No Baker's cyst.

### CONCLUSION:

Severe insertional semimembranosus tendinopathy, with a small peritendon ganglion cyst. No internal joint derangement. No Baker's cyst.

## MRI CERVICAL SPINE

### HISTORY:

Neck pain since few weeks restricted mobility to right side associated slight paraesthesia and arm. Rule out radiculopathy. Known high uric acid levels.

### TECHNIQUE:

Non-contrast MRI performed on 3T platform

### FINDINGS:

Vertebral body height is preserved throughout the cervical spine. No crush fracture or destructive osseous lesion. The cervical medullary junction is normal. Cervical spinal cord is normal in appearance with no features of myelomalacia and no neural lesion.

There is very minor disc desiccation at the C5/C6 level. No significant focal disc protrusion at any level of the cervical spine. No significant spinal canal or neural foraminal stenosis. The facet joints are normal.

No cervical ribs. The paravertebral soft tissues are normal.

### CONCLUSION:

No evidence of neural impingement within the cervical spine.

## DUAL ENERGY CT RIGHT ANKLE AND GREAT TOE

History: Exclude urate deposition or extra-articular structures.

Technique: Non contrast dual energy CT with 3D reformats.

Findings: Monosodium urate crystal deposition is demonstrated at the great toe interphalangeal joint, 2nd toe PIP and DIP joints and 3rd toe PIP and DIP joints. There is no focal bony erosion demonstrated. No peri-articular soft tissue calcification is appreciable on conventional CT imaging.

Joint alignment is maintained. The joint spaces are relatively preserved with no evidence of established degenerative osteoarthritis. No periosteal reaction. A prominent great toe medial subungual osteophyte is noted.

Conclusion: Dual energy CT demonstrates monosodium urate crystal deposition involving the great toe interphalangeal joint and 2nd and 3rd toe PIP and DIP joints.

EXAMINATION: XRAY LEFT HAND; XRAY RIGHT HAND

Clinical History: with 1 history of oligo arthropathy. Exclude erosive arthritic change.

Findings: No erosive changes are present at the radiocarpal, intercarpal, carpometacarpal, metacarpophalangeal and interphalangeal joints of the hands. Joint spaces are normal.

No soft tissue calcifications are present.

Bony trabecular pattern is normal bilaterally.

## MRI RIGHT ANKLE

### Clinical Details:

? Talonavicular ligament tear.

### Findings:

AITFL, PITFL and PTFL are intact.

ATFL is oedematous with preserved fibrillar pattern, compatible with mild sprain injury.

Intact CFL.

Satisfactory peroneus tendons.

Anterior tibiotalar and deep posterior tibiotalar ligaments are satisfactory.

Tibiospring ligament is intact.

Tibialis posterior, FDL and FHL tendons are satisfactory.

Tibialis anterior, EDL and EHL tendons are satisfactory.

Inferior extensor retinaculum is intact.

Congruent tibiotalar joint. No osteochondral lesion.

Subtalar, Chopart and Lisfranc joints are satisfactory.

Dorsal talonavicular ligament, bifurcate ligament, dorsal calcaneal ligament and spring ligaments are satisfactory.

Intact Achilles tendon.

Satisfactory plantar fascia.

### Conclusion:

ATFL mild sprain injury. No high ankle sprain.

History: Chronic pain right elbow and shoulder.

#### MRI RIGHT SHOULDER

Technique: Multiparametric MRI right shoulder.

Findings: The rotator cuff is intact. Rotator cuff muscle bulk is normal.

Long head biceps tendon is intact. There is hypointense tissue within the inferior glenohumeral joint space (see key image). A suspected tear of the anteroinferior labrum is present with poorly defined labrum which demonstrates increased signal. The tissue is most likely a flipped labral fragment. There is a further tear involving the posterosuperior labrum which is attenuated and of increased signal.

Glenohumeral cartilage is maintained. AC joint appears normal. Trace increased fluid within the subacromial/subdeltoid bursa.

Conclusion: Suspected anteroinferior labral tear with displaced labral tissue in the inferior glenohumeral joint. Further small chronic appearing posterosuperior labral tear is present.

#### MRI RIGHT ELBOW

Technique: Multiparametric MRI right elbow.

Findings: Low grade tendinosis of the common extensor origin with a very small intrasubstance tear involving the deepest fibres. Tear cleft is very linear measuring approximately 3mm in length. The radial collateral ligament is poorly defined towards the humeral attachment but thought to be grossly intact. Collateral ulnar collateral ligament is intact.

Common flex origin and ulnar collateral ligament is intact.

Biceps, triceps and brachialis appear satisfactory.

Articular cartilage of the elbow is maintained. No effusion. No loose bodies.

Normal appearance of the median, radial and ulnar nerve.

Conclusion: Low grade tendinosis of the common extensor origin with minor intrasubstance tearing.